Lecture 19: What Happens When Things Go Wrong: Mental Illness, Part II

Introduction to Psychology · Yale University, Spring 2007 · Professor Paul Bloom

Study guide based on the lecture transcript. Source: https://oyc.yale.edu/psychology/psyc-110/lecture-19 (CC BY-NC-SA 3.0).

Before you start

This lecture continues the topic of clinical psychology, also called abnormal psychology or psychopathology. Professor Bloom first asks how we should define mental illness. He then reviews four groups of disorders (schizophrenia, anxiety disorders, dissociative disorders and personality disorders) and ends with a short history of therapy.

Learning goals

After this lecture you should be able to:

- Explain the debate about whether mental illness is a real illness or a social label.
- Describe the positive and negative symptoms of schizophrenia and its possible causes.
- Compare three anxiety disorders.
- Describe the three dissociative disorders and explain why one of them is controversial.
- Explain why antisocial personality disorder is a "hard case".

Key ideas

1. Identifying Mental Illness

- By one estimate, half of all college graduates in the United States will have a mental illness in their lives serious enough to need treatment.
- Some cases are hard to judge: is a killer without a conscience, or a mobster like John Gotti, mentally ill? Is a man who gives a kidney to a stranger?
- The psychiatrist Thomas Szasz claimed that calling someone mentally ill is a social decision, not a medical one: a way to **ostracize** (push out) people who differ from society's norms. Bloom says this is not entirely unreasonable: slaves in the United States who tried to escape were described as mentally ill, and homosexuality counted as a mental illness until 1973.
- But people go too far when they say mental illness does not exist. With very few exceptions, it damages people's functioning, and people who get better do not choose to go back. Today, "Schizophrenia is as much a disease as is cancer."

2. Schizophrenia

- About 1% of the world's population has schizophrenia, the most common reason for being in a mental hospital. The name comes from "split" and "mind", but it means a split from reality, not multiple personalities.

- It has four **positive symptoms** (unusual things a person has or does): **hallucinations** (false sensory experiences, usually hearing voices), **delusions** (false beliefs, such as "ideas of reference": thinking that things happening around you are about you), disorganized speech, and disorganized behavior (for example, being **catatonic**: not moving). The **negative symptom** is an absence of normal thought or **affect** (emotion). Three major subtypes are paranoid, catatonic and disorganized.
- The core problem may be an inability to put thoughts and perceptions together in a logical order. This leads to less social contact and less "reality checking": a vicious circle.
- If your identical twin is schizophrenic, your odds are about one half. So genes make people vulnerable, but the environment matters too. Possible triggers include trauma at birth and viral infections (more schizophrenics are born in winter). Stressful families are linked to schizophrenia, but cause and effect are unclear.
- The old theory of too much **dopamine** (a brain chemical) is incomplete: for example, it does not explain the negative symptoms. A mystery: less industrialized countries have better recovery rates, "and nobody really knows why."

3. Anxiety Disorders

- Everybody feels anxiety. It becomes a disorder when it is too much, uncontrollable and unreasonable, and it messes up your life.
- **Generalized anxiety disorder** affects about one in twenty people at some point in their lives: they worry all the time. In one model, a bad childhood experience makes a person **hyper-vigilant** (always watching for danger).
- **Phobias** are intense, irrational fears. About 40% of people say they are afraid of snakes, but few fear cats. The classical conditioning model (fear learned from a bad experience) is weak, because many people afraid of snakes never had a bad experience with snakes. The "**preparedness theory**" says we evolved to fear things that were dangerous in our evolutionary history.
- **Obsessive-compulsive disorder (OCD)** affects about two to three percent of people. **Obsessions** are irrational, disturbing thoughts; **compulsions** are repeated actions to reduce them, such as compulsive washing. OCD is treated quite well with drugs that affect serotonin, a brain chemical.

4. Question and Answer on Schizophrenia and Anxiety Disorders

Some disorders are "**comorbid**" (they often go together), and superstitions can creep into OCD when they become rituals. Bloom said that people with schizophrenia are statistically more likely to be victims than harmful.

5. Dissociative Identity Disorders

- **Dissociation** means becoming separated from some part of your identity or memory. Some dissociation is normal, such as driving a car and not remembering part of the trip.
- **Dissociative amnesia** is memory loss after something terrible. **Dissociative fugue** is a loss of all past memory plus a new identity: a man whose wife left him was found six months later tending bar in Miami Beach, calling himself Martin.

- **Dissociative identity disorder**, once called "multiple personality disorder", means two or more distinct people in one head. It is often argued to be caused by severe abuse. One idea is abuse plus a tendency to dissociate; in fact, these patients are easier to hypnotize.
- It is very controversial. Less than one quarter of psychiatrists believe it exists. There were two US cases between 1930 and 1960 but 20,000 in the 1980s, and some therapists see it in almost every patient. One worry is that the therapist's suggestion creates it.

6. Question and Answer on Dissociative Identity Disorders

Unlike amnesias caused by brain damage, dissociative amnesia is caused by life events and is often short-lived. Bloom thinks the famous case of Sybil, made into a movie, had a huge influence on people.

7. Personality Disorders

- In a **personality disorder**, a personality is so extreme that it becomes a mental illness. Examples include narcissistic, avoidant, borderline and paranoid personality disorders.
- Most murderers are not mentally ill in a medical sense; they are driven by normal desires, such as rage or jealousy, taken to an extreme.
- **Antisocial personality disorder** (once "moral insanity", now often "**psychopathy**") describes people, typically male, who are selfish, callous (without feeling for others) and impulsive, lack love, loyalty and compassion, and are easily bored. Bloom jokes that James Bond, as played by Sean Connery, is a perfect psychopath.
- Is it an illness? Psychopaths do not come in for treatment, and many are reasonably successful. Psychologists study only unsuccessful psychopaths; some argue that the successful ones run the world.

8. Brief History on Therapy

The key question is "Does therapy work?" The history of therapy has been "gruesome and unsuccessful": mad people were seen as working with the devil and were tortured, burnt or sent out to sea. In the nineteenth century, Pinel tried to have mental hospitals. Bloom wonders whether, a hundred years from now, our current therapies will look just as barbaric.

Key vocabulary

Term	Meaning in simple English	Example from the lecture
ostracize	to push someone out of a group or society	Szasz: mental illness labels ostracize people
hallucination	a sensory experience that is not real	Hearing voices that tell you to do things
delusion	a belief that is clearly false	Believing you are Jesus Christ
affect	emotion	Some schizophrenics have very low emotional responses
preparedness theory	we evolved to fear things that were dangerous in the past	Fear of snakes

Term	Meaning in simple English	Example from the lecture
obsession / compulsion	a disturbing thought / a repeated action to reduce it	Feeling dirty / compulsive washing
comorbid	often occurring together	Severe depression plus an anxiety disorder
dissociation	separation from part of your identity or memory	Not remembering part of a car trip
dissociative fugue	memory loss plus a new identity	The man who became "Martin" in Miami Beach
psychopathy	another name for antisocial personality disorder	James Bond, as played by Sean Connery

Check your understanding

1. What did Thomas Szasz claim, and what is one example that supports his view?
2. Why does Bloom think it goes too far to say mental illness does not exist?
3. Is schizophrenia the same as "split personality"? Explain.
4. What is the difference between a hallucination and a delusion?
5. What does the "about one half" rate for identical twins tell us about schizophrenia?
6. Why is the preparedness theory better than the classical conditioning model of phobias?
7. Give two reasons why some experts doubt dissociative identity disorder.
8. Why is antisocial personality disorder a "hard case"?

Answer guide

1. Calling someone mentally ill is a social decision to ostracize people who differ from norms. Example: homosexuality counted as a mental illness until 1973.
2. Real mental illness damages functioning, and people who get better do not choose to go back to it.
3. No. It means a split from reality; multiple personalities are a different, dissociative disorder.
4. A hallucination is a false sensory experience; a delusion is a false belief.
5. If it were entirely genetic, the rate would be 100%, so the environment matters too.
6. Many people fear snakes without any bad experience with them; we seem to have evolved to fear old dangers.
7. Cases jumped from two (1930-1960) to 20,000 in the 1980s, and some therapists see it in almost every patient. It may come from the therapist's suggestion.
8. Psychopaths do not seek treatment, and many are successful. Other people have a problem with them, but that may not make it a disorder.

English practice

Useful phrases from the lecture

- "Now, this is not entirely an unreasonable view." - partly agrees with an idea.
- "People go too far when they say..." - says a view is too extreme.
- "It's hard to pin down exactly what's at root of all of these problems." - says a cause is difficult to find.
- "It's not clear whether the effect is from... to... or from... to..." - questions the direction of cause and effect.

Speaking task (with your AI tutor)

- Explain the symptoms of schizophrenia in 4-6 sentences. Use: *hallucination*, *delusion*, *affect*.
- Explain why dissociative identity disorder is controversial in 4-6 sentences. Use: *suggestion*, *therapist*, *cases*.

Writing task

- Write one paragraph (80-120 words): "Is mental illness a real illness or a social label?" Summarize Szasz's view and Bloom's response, with one example.

Study with your tutor

Things to say to your tutor in Live conversation:

- "Let's review Lecture 19. Ask me questions about schizophrenia, one at a time."
- "Quiz me on the key vocabulary of Lecture 19."

- "I will explain the three dissociative disorders in my own words. Check my understanding and my English."

Readings from the original course

- Gray, Peter. Psychology (5th edition), chapter 16
- Kramer, Peter D. "Listening to Prozac: A Psychiatrist Explores Antidepressant Drugs and the Remaking of the Self." In The Norton Psychology Reader. Edited by Gary Marcus. New York: W. W. Norton & Company, 2006. pp. 366-379
- Beck, Aaron T. "Cognitive Therapy and the Emotional Disorders." In The Norton Psychology Reader. Edited by Gary Marcus. New York: W. W. Norton & Company, 2006. pp. 380-386

My notes